INFLIXIMAB

Infusion reactions

Screen for TB & hepatitis B

CHEMICAL NAME Chimeric (mouse–human) IgG1κ monoclonal antibody against TNF-α
CLASS Biologic — TNF-α inhibitor (chimeric monoclonal antibody)
TRADE NAMES Remicade · Inflectra · Remsima (incl. SC) · Flixabi

THERAPEUTIC USES / INDICATIONS

- Crohn's disease (incl. fistulising) and ulcerative colitis (incl. acute severe UC rescue).
- Rheumatoid arthritis (with methotrexate), ankylosing spondylitis, psoriatic arthritis.
- Plaque psoriasis; paediatric IBD.

ADMINISTRATION

- **IV infusion over 2 h** (later infusions may be shortened to 1 h if tolerated) using 0.2-micron in-line filter.
- Premedication (paracetamol, antihistamine ± hydrocortisone) per protocol.
- Observe for 1–2 h after infusion (first infusions).
- SC formulation (Remsima SC) for maintenance.

DOSAGES

- **IBD:** 5 mg/kg IV at weeks 0, 2 and 6, then every 8 weeks (may escalate to 10 mg/kg or every 4–6 weeks).
- **RA:** 3 mg/kg at weeks 0, 2, 6, then every 8 weeks (with methotrexate).
- **Acute severe UC rescue:** 5 mg/kg (accelerated dosing).

PHARMACOKINETICS

- **Onset:** IBD response within 2 weeks.
- Catabolised like IgG.
- **t½:** 8–9.5 days.

PHARMACODYNAMICS

- Binds **soluble and transmembrane TNF-α** → neutralisation, apoptosis of TNF-producing cells → ↓ inflammation, mucosal healing.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Infusion reactions: fever, chills, urticaria, flushing, headache.
- Upper respiratory infections.
- Nausea, abdominal pain.

ADVERSE EFFECTS (SERIOUS)

- Anaphylaxis; delayed hypersensitivity (serum-sickness-like, 3–12 days).
- Serious infections, TB, hepatitis B reactivation.
- Hepatosplenic T-cell lymphoma (young males with thiopurines); other malignancies.
- Heart failure worsening; demyelination; antibody formation (loss of response).

CONTRAINDICATIONS & CAUTIONS

- Active TB, severe infections.
- Moderate–severe heart failure.
- Hypersensitivity to murine proteins.
- Live vaccines.

KEY INTERACTIONS

- **Thiopurines (azathioprine):** ↑ infection, lymphoma risk (but ↓ antibodies).
- **Live vaccines:** contraindicated (also in infants exposed in utero for 6–12 months).

NURSING MANAGEMENT

- Pre-infusion checklist: screening, current infection, vital signs, weight.
- Vital signs every 30 min during infusion and after; slow/stop for reactions; anaphylaxis kit ready.
- Use filter set.
- Document infusion schedule; monitor drug levels/antibodies if loss of response.

PATIENT EDUCATION

- Report fever, chills, rash or breathing problems during or after the infusion.
- Report joint pain, rash and fever 3–12 days after (delayed reaction).
- Report infections early; avoid live vaccines.
- Baby exposed in pregnancy should avoid live vaccines for up to 12 months.

KEY POINTS

- Chimeric anti-TNF antibody; IV infusion.
- Infusion reactions; filter set.
- IBD: 5 mg/kg at 0, 2, 6 weeks then 8-weekly.
- Screen TB/HBV; serious infections.